coursera

Depressive and Bipolar Disorders ×

Today's goal • 3/3 items

Module 1
Learn With PsycLearn Essentials

Module 2
Opening to Depressive and Bipolar Disorders

Module 3
Depressive and Bipolar Disorders

Module 4
Biological Contributing Factors

Lesson 5: Genetic Contributing Factors
- Developmental Psychopathology
  Reading • 3 min
- Genetic Contributing Factors
  Reading • 3 min
- The Link Between Genes and Psychopathology
  Video • 4 min
- Practical Scenario
  Real-world application • 5 min
- Genetic Contributing Factors for Depression
  Reading • 10 min
- Epigenetic Contributors to Depression
  Video • 3 min
- Genetic Contributing Factors for Bipolar Disorder
  Reading • 3 min
- Check Your Understanding: Genetic Contributing Factors
  Practice Assignment • 4 min

Lesson 6: Neurobiological Contributing Factors
- Depressive Disorders: A Case Study
  Reading • 5 min
- Communication in the Synapse
  Video • 4 min
- Neurochemical Contributors to Depression
  Reading • 10 min
- Practical Scenario
  Real-world application • 5 min
- Hormonal Contributors to Depression
  Reading • 5 min
- Brain Regions Linked to Depression
  Video • 4 min
- Neurobiological Contributors to Depressive and Bipolar Disorders
  Reading • 15 min
- Check Your Understanding: Biological Contributing Factors
  Practice Assignment • Grade: 83.33%

Key Takeaways
- Key Takeaways: Biological Contributing Factors
  Reading • 10 min

Module 4 Quiz
- Module Quiz: Biological Contributing Factors
  Graded Assignment • Grade: 88.88%

Module 5
Psychological Contributing Factors

Lesson 7: Psychological Contributing Factors
- Psychological Contributing Factors
  Reading • 10 min
- Check Your Understanding: Psychological Contributing Factors
  Practice Assignment • 5 min

Lesson 8: Stress and Mood Disorders
- Stress and Mood Disorders
  Reading • 15 min
- Check Your Understanding: Stress and Mood Disorders
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Key Takeaways
- Key Takeaways: Psychological Contributing Factors
  Reading • 10 min

Module 5 Quiz
- Module Quiz: Contributing Factors II
  Graded Assignment • 10 min

Module 6
Treatments for Depressive and Bipolar Disorders

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Psychological Contributing Factors

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There is an interplay among the biological and psychological factors that influence depressive and bipolar disorders. The **developmental psychopathology** approach to explain psychological disorders (Cicchetti, 2013) examines the origins and progression of psychological disorders using a framework that focuses on the typical processes of human growth and maturation. At every stage we experience biological, social, cognitive, emotional, and psychological challenges. Developmental psychopathologists believe it is important to consider all these factors in explaining behavior; no single factor in isolation is responsible for causing psychological disorders (Cicchetti, 2013; Cicchetti & Rogosch, 1996). In the previous section, we examined biological factors, such as genetics, neural activity, hormone or neurochemical levels, or broader environmental factors. In this and the following section, we will examine psychological factors, such as cognitive distortions, feelings of helplessness or hopelessness, ruminating behaviors, interpretations of ambiguity, and how stress is associated with mood disorders. The salient point is that biological factors interact with psychological factors, but the various psychological factors also interact with each other (LeMoult, 2020).

Psychological Factors and Depression

In the 1940s and 1950s, the most popular approach in explaining human behavior was behaviorism, which focused only on external, observable phenomena such as overt behaviors. By the 1960s, however, many psychologists were beginning to examine how internal, cognitive phenomena, such as thoughts, memories, and feelings could both help and hurt mental health. Cognitive psychology grew from the observations that how humans process, think about, and remember information can be used to understand human behavior.

The Cognitive Triad

A prominent researcher investigating the link between cognition and mental health was the psychiatrist Aaron Beck who noticed how both he and his patients experienced automatic thoughts in response to life events (Beck, 1963, 2019). **Automatic thoughts** are instantaneous, habitual, and nonconscious thoughts that may be positive or negative. Beck recognized that the automatic thoughts of many of his clients with depression were negative. **Negative automatic thoughts (NATs)** are instantaneous, habitual, and self-critical. When these negative automatic thoughts involve exaggerations, misinterpretations, and distortions of an event then we refer to them as **cognitive distortions.** They are rooted in our core beliefs about the self, the world, and the future, which Beck called the **cognitive triad.**

The cognitive triad is part of a person's larger system of beliefs, and affects how people respond to setbacks and challenges. Research has shown that depression is more common in those who consistently respond negatively to frustrating events and losses (Lahey, 2009; Lim et al., 2018). Beck suggested that if a person had only one type of negative belief, say negative beliefs about the self, but not about the world or the future, they would have some hope that their life circumstances could improve in the future. When all three sets of automatic thoughts are negative, people feel change is hopeless, and as a result are unmotivated to try to improve their situation.

Examples of events and possible negative automatic thoughts are shown in the following table, titled *Events and Negative Automatic Thoughts*.

Events and Negative Automatic Thoughts

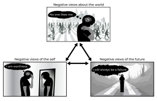
Event	Possible Negative Automatic Thoughts
Your car breaks down.	Nothing ever goes right for me, and it never will.
You meet a new person.	They didn't smile at me when we met. They don't like me.
You get a poor result on an exam.	I knew I was going to fail. I will never be successful.

Beck's work evolved into a **generic cognitive model** that has been applied to understanding many types of psychological disorders (Beck & Haigh, 2014). The basic idea behind the generic cognitive model is that psychological disorders result from an extreme version of adaptive thinking. Experiencing a variety of emotions, such as happiness, anger, sadness, and anxiety, is normal and adaptive; however, when people experience emotions to an extreme degree, often because of cognitive biases, they are at risk for psychopathology (Beck & Haigh, 2014).

Beck believed that faulty or biased information processing affects our emotions, behaviors, and motivation (Beck, 1963; Neisser, 1967). When this faulty reasoning is extreme enough, we are at risk for a psychological disorder, such as depression or anxiety. The most damaging beliefs are those that are deep-seated and often come down to core ideas like "I am a failure" or "I don't deserve to get better." Such beliefs can emerge as conscious thoughts or as automatic, mostly unconscious, thoughts. Someone with the belief that they are a failure may automatically think "I'm going to mess this up" when asked to do a particular task. That faulty belief may cause the person not to give the task their full effort since they feel they are going to do poorly anyway, and then a failure confirms the negative self-belief. This creates a loop where a negative view of the self affects the person's view of the world that affects the person's view of the future, which feeds back on the person's view of self.

The following *Cognitive Triad* image describes how negative thought processes in the cognitive triad influence each other.

Cognitive Triad


Negative views about the world
No one likes me
Negative views of the self
I am worthless
Negative views of the future
I will always be a failure

Note. The cognitive triad illustrates how negative thought processes interact to influence each other.

Role of Attention, Memory, and Ambiguity on Mood Disorders

The way we think about the world can both cause and reinforce depressive episodes common to both depressive disorders and bipolar disorders. More recent research has shown that, in addition to negative thoughts about the self, a wide range of cognitive processes influence the onset and course of depression, including attention, interpretation, and memory (Castaneda et al., 2008; Kircanski et al., 2012; LeMoult & Gotlib, 2019).

For example, there is mounting evidence that depression is associated with dysfunction in attentional processes, specifically, individuals who are depressed appear to have difficulty drawing their attention away from negative stimuli, such as sad faces, to focus on neutral stimuli (Sanchez et al., 2013). This difficulty in shifting attention by people with depression appears to play a role in the maintenance of sad moods (Sanchez et al., 2013). Also, people with depression are more likely to interpret ambiguous information in negative ways (Lee et al., 2016). Once depressed people make negative interpretations, those interpretations are encoded in their memory (Hertel & Brozovich, 2010). When combined with the evidence that individuals who are depressed exhibit mood-congruent memory, which means that they remember more negative events and information than nondepressed participants (Gotlib et al., 2011; Matt et al., 1992), this creates an interaction where attention to negative stimuli is misinterpreted and then encoded and recalled disproportionately.

In large part, the impact that our thoughts can have on depression arises because much of what we experience every day is ambiguous. Many events that we experience or think about can be interpreted in a variety of ways. People with depression tend to interpret these ambiguous events more negatively than others (Andersen & Schwartz, 1992; Everaert, 2021). This processing difference both reveals the factors contributing to depression and suggests ways that it can be treated.

Helplessness Theory of Depression

It's often hard to accurately know what is causing negative events in our lives. If we make the wrong guesses about why something is happening, the steps we take to try to stop it won't work, raising the possibility that we will come to believe we can't do anything to change what's happening to us. A person who goes on failed date after failed date may assume that they never work out because of their physical appearance. As a result, they may spend a large amount of time and effort trying to improve their look. However, if they make a wrong assumption that their appearance was the problem—perhaps they talked about themselves too much—none of the steps they take will work. This may lead to the person giving up trying to improve their dates, or even stopping dating. Depression is a potential result of this learned helplessness.

Helplessness theory (or learned helplessness) theorizes that when people are not able to exert control in their lives, this may lead to a belief that they are helpless (Maier & Seligman, 1976, 2016). These experiences of helplessness communicate to people that nothing they could do matters, and in future situations, then, even when they could exert some control over a situation, they do not make the effort (Maier & Seligman, 2016). Feeling helpless has been linked to the majority of depressive symptoms, including feelings of sadness, loss of interest in usual activities, changes in sleep and appetite, and feelings of worthlessness (Maier & Seligman, 2016). However, helplessness theory does not explain how some people are able to experience loss of control but do not develop depression.

Hopelessness Theory of Depression

To address gaps in helplessness theory, Abramson and colleagues (1978, 1989) proposed the **hopelessness theory** and hypothesized that people developed depression if they attributed a negative event (such as a poor grade on a test) to causes that were internal ("I'm unintelligent"), stable ("I'll always be unintelligent"), and global ("I'm a failure in all areas of my life, not just on this test") as opposed to blaming it on external ("It was a hard test"), specific ("I didn't get enough sleep last night"), and unstable ("I don't usually stay up to cram before an exam") stimuli. According to the hopelessness theory, these negative cognitive styles interact with negative life events to produce hopelessness and contribute to depression (Liu et al., 2015).

As an opportunity to explore your beliefs about whether you have control over the outcomes of your life (i.e., an internal locus of control) rather than external forces (i.e., an external locus of control), complete the following worksheet, *The Internal-External Locus of Control Worksheet*. For each statement, place an X in one column to record the extent to which the statement personally applies to you. Use a scale from 1 to 5, where 1 = "Does not apply at all" and 5 = "Applies completely" to you. For example, if you think the statement "I'm my own boss" applies somewhat to you then you would score a 3 for that statement. This scale is based on the work of Nießen and colleagues (2022). Following this table is a link to a downloadable worksheet of the activity formatted for Microsoft Word.

The Internal-External Locus of Control Worksheet

	1. Does not apply at all	2. Applies a bit	3. Applies somewhat	4. Applies mostly	5. Applies completely
1. I'm my own boss.					
2. If I work hard, I will succeed.					
3. Whether at work or in my private life: What I do is mainly determined by others.					
4. Fate often gets in the way of my plans.					

Note. Blank worksheet for the Internal-External Locus of Control Short Scale.

DBDCAM04L07DL001 The Internal-External Locus of Control Worksheet
DOCX File

Now that you've completed the table, you can calculate your internal locus and external locus scores. To estimate your internal locus of control (i.e., the degree to which you have internal control of your life), calculate the average points for the first two statements. Next, to estimate your external locus of control (i.e., the degree to which external forces determine your life), calculate the average points for the last two statements. The higher of the two scores indicates your perception, or locus, of control.

As an example, review the averaged scores from a sample of 468 participants (222 men; 246 women) from the United Kingdom and 474 participants (237 men; 237 women) from Germany (Nießen et al., 2022). Notice that for both men and women, the average scores for the locus of control was higher for internal than for external control, suggesting that, on average, this sample of UK and German residents felt in control of their lives.

The Internal-External Locus of Control Example

Locus of Control	United Kingdom		Germany	
	Men	Women	Men	Women
Internal	3.37	3.43	3.99	4.06
External	2.49	2.43	2.48	2.55

Note. Example worksheet for the Internal-External Locus of Control Short Scale.

Rumination and Depression

There is a tendency of people with depression to engage in rumination. **Rumination** is obsessional thinking involving excessive, repetitive thoughts or themes that interfere with other forms of mental activity. It has also been noted that it involves "focusing on symptoms of distress and on the possible causes and consequences of these symptoms" (Nolen-Hoeksema et al., 2008, p. 400). That is, it is a sort of wallowing in distress and depression. People who ruminate remain focused on the problems they are experiencing, rather than on active problem-solving to deal with the problem and prevent it from recurring (Spasojevic & Alloy, 2001). Rumination has long been associated with both the beginning and continuation of depressive episodes (e.g., Zhou et al., 2020).

For example, imagine a person who has been called out at work for dozing off. If the person doesn't ruminate very much, they might feel embarrassed at the time and make a plan so that it doesn't happen again, like making sure they get enough sleep or drink caffeine before work. However, if they have a tendency to ruminate, the feelings of embarrassment for themselves or anger at their boss or self-recrimination for falling asleep might persist for days, weeks, or even months after the event. Note that in this case of rumination, not only are there prolonged negative emotions, but the worker also lacks a plan to prevent falling asleep at work in the future.

Research suggests that the relationship between rumination and depression may be bidirectional. Rumination may predict depression and depression may predict rumination (Krause et al., 2018). Psychologist Elizabeth Krause and her colleagues examined depression and rumination in a sample of 408 adolescent boys and girls. As they hypothesized, there was a cycle between rumination and depression for girls. However, for boys, depression led to rumination but rumination did not predict depression (Krause et al., 2018). However, in a recent longitudinal study of 5891 adults, Mark Whisman and colleagues (2020) found that gender, as well as age and neuroticism, did not explain the bidirectional association between rumination and depression. Their research supports the finding that, at least among adults, rumination is both a cause and a consequence of depression.

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